SAMPLE ANSWER · CEREBRAL PALSY · CASE 2

Sample 7-minute ISBAR: Kai

Kai Watson · 9-year-old boy · Outpatient paediatric occupational therapy service · about 1039 spoken words (≈ 7.4 min at 140 words per minute)

I Introduction

60 words

"Hi, I'm the occupational therapy student running Kai Watson's constraint-induced movement therapy session this morning. Kai is a 9-year-old boy with left spastic hemiplegic cerebral palsy. I need your help urgently because I think there's a problem with the circulation and nerves in his right hand under the constraint cast, and I believe it needs checking and probably removing now."

S Situation

138 words

"Kai's cast was applied to his right forearm and hand five days ago. Today his right fingertips are puffy and pale, with dusky, bluish nail beds. They're cooler than his left fingers, capillary refill is slow, and he can only wriggle them slowly. He describes pins and needles and numbness, he winces when I gently straighten his fingers, and he says the pain kept him awake last night.

"His dad, Ben, told me he trimmed some padding out of the cast with scissors last night because it was rubbing, then taped it back down. There's a lot of packing tape wound firmly round the wrist end. Both parents think Kai is exaggerating to avoid work. The next family is waiting and Rachel would like a video before they leave for school at ten, but I've stopped the activity."

B Background

134 words

"Kai lives in Townsville with his parents, Rachel and Ben, and is in Year 4. He's GMFCS level I and MACS level II, so he walks and runs independently and uses his left hand mainly as a helper. He's in week one of a three-week modified CIMT programme funded through his NDIS plan, focused on his goals of building Lego and tying his shoelaces.

"When the cast was applied, it was documented as well fitted, with warm, pink fingers and normal sensation. His home diary shows every practice session completed.

"There are gaps I'd need to clarify. There's no documentation that the family received written cast-care instructions, that the cast has been checked since it was applied, what time his symptoms started, whether he's had any pain relief, or how much padding was removed."

A Assessment

311 words

"My main concern is that Kai's signs — pale and dusky fingers, coolness, slow capillary refill, swelling, pins and needles, numbness, reduced movement and pain that's woken him at night — together may indicate neurovascular compromise under the cast. Removing padding and retaping can make a cast tighter or create a pressure point, and swelling inside a rigid cast can't expand outward. If pressure continues, there's a risk of nerve, skin and circulation damage, and in rare cases a serious rise in pressure within the forearm. That needs action now, not at the end of the session.

"Although his cast was documented as well fitted five days ago, that doesn't tell us about the cast today, after it's been altered. And the full home diary shows commitment, not safety.

"I don't think this is Kai avoiding work. He's been motivated, he wants to do the Lego, and his complaints match what I can see. Children can find sensations hard to describe, so his report deserves to be taken seriously.

"Occupationally, the casted right hand is his more skilled hand, the one he relies on for writing, eating and self-care. Any lasting injury there would cost him far more function than the programme could gain, so protecting it comes before therapy dosage.

"The other requests are understandable but not priorities now. The video and the NDIS report can wait. Ben's idea of more hours is well meant, but I wouldn't change the dose, which was set deliberately, and certainly not while there's a cast problem. Camp next week is something I'd factor into planning later.

"The home alteration also suggests our cast-care education may not have been clear enough.

"So my immediate priority is cast assessment and removal or release according to our protocol, followed by medical review if his signs don't fully resolve. The programme and family requests come after that."

R Recommendation

396 words

"My immediate recommendation is that I stop the session, keep Kai resting with his right arm supported at about heart level, and ask you, or the clinician trained in cast removal, to come now to check the cast and release or remove it under our service's protocol. I'll repeat neurovascular checks every few minutes — colour, warmth, capillary refill, sensation, movement and pain — and record the times and findings.

"If his circulation and sensation don't return promptly once the cast is off, or if his pain is out of proportion, I'd like your help to get him to the emergency department straight away, because that could mean rising pressure within the forearm. If symptoms resolve, I'd still recommend medical review to check for pressure areas or nerve irritation, and I'll complete an incident report.

"I'd speak to Rachel and Ben calmly, explaining what we've found and why it matters, without blaming Ben, who was trying to help. I'd also tell Kai that we believe him and that he did the right thing telling us.

"I'd let reception know the next family will be delayed, and suggest Rachel films a short clip of a later session instead.

"For the team, I'd ask medical review to assess circulation, nerve function and skin, and the person who applied the cast to review why it became tight. If the programme continues, I'd discuss with you whether to use a removable splint or a different constraint method.

"For OT, my principal short-term assessment would be a skin and neurovascular check of the right hand once the cast is off, followed by observed fine motor performance with his right hand, such as picking up Lego pieces, to make sure function has returned. For the longer term, I'd use the Assisting Hand Assessment, which is designed for children with unilateral cerebral palsy, to measure change in his left hand's contribution to two-handed tasks, alongside Kai's own goals.

> "I'd provide written cast or splint care instructions, including what to check, when to call us and what not to do at home, and teach both parents and Kai to recognise warning signs. CIMT has strong evidence in cerebral palsy, but safe constraint is part of the programme.
>
> "Finally, I'd follow up with the family by phone this afternoon, and talk with Kai about his goals for camp and his shoelaces, so he stays motivated."

Coaching notes *not spoken*

The immediate priority

Kai shows clear signs of neurovascular compromise under a cast that a parent has altered at home. The cast must be checked and released or removed now under the service protocol, with ongoing neurovascular observations and medical review if signs persist.

Distractors in this case

- **Cast documented as "well fitted" on day one** — five days and a home alteration later, that finding is out of date.
- **Full home diary and an enthusiastic child** — motivation does not mean the constraint is safe.
- **Parents' view that Kai is "a drama king"** — his report matches objective signs; believe the child.
- **Video for Grandma and the NDIS report, extra hours, school camp, waiting family** — all can wait; more dose is not a fix for a safety problem.

Missing information to clarify

- Time of onset and progression of symptoms
- Pain relief given overnight
- What cast-care education the family received
- Exactly how the cast was altered

Useful references

- Novak I, et al. (2020). State of the evidence traffic lights 2019: systematic review of interventions for preventing and treating children with cerebral palsy. *Current Neurology and Neuroscience Reports*, 20(2), 3.
- Assisting Hand Assessment (AHA) manual, Krumlinde-Sundholm and colleagues.

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